



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : صفاء فتوح السيد سليمان
المحافظة : القاهرة
الحى / المدينة : عين شمس
رقم البطاقة : 27501010105863 رقم التليفون : 01125811104
العنوان : =
العمر (تاريخ الميلاد) : 01-01-1975 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 05-10-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: صفاء فتوح السيد سليمان National ID: 27501010105863	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
مفاتيح



Name : Safaa Fattouh Al Sayed
Branch : Portable
Ref.Physician Prof : CAP+C

Date : 16-02-2026 01:40
Medical No. : GRI-12243
Ins.Company :

**MULTI-DETECTOR C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS FOLLOWING ORAL AND
INTRAVENOUS CONTRAST MEDIA ADMINISTRATION
REVEALED:**

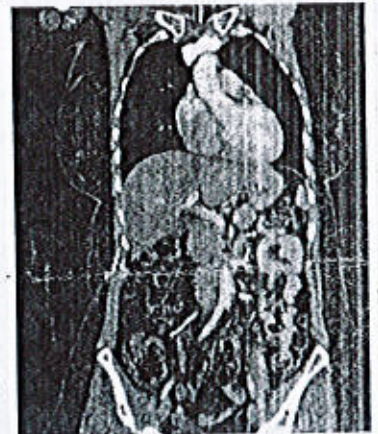
Clinical data: Breast cancer as compared with previous CT dated 7/10/2025 the current study reveled

Stationary course with no interval changes

- Normal lung parenchyma, showing no focal lesions.
- Normal vascular mediastinal structures.
- The trachea, main stem bronchi as well as their lobar and segmental divisions are normal.
- No evidence of mediastinal or hilar nodal enlargement .
- No pleural collections.
- No detectable lesions in the chest wall.
- The liver is of normal size, and tissue density. It shows no focal or enhancing lesions.
- The intra and extra-hepatic biliary radicles are normal.
- The pancreas and spleen are normal.
- Normal CT features of both kidneys.
- Right and left supra renal glands are normal.
- No CT evidence of enlarged abdominal or pelvic lymph nodes.
- No ascites.
- Scans through the pelvis showed no masses.
- No suspicious bony lesion

Opinion

Stationary course with no metastatic deposits



**Much Obliged;
Dr.Khaled Moghazy,MD
Prof.of Radiology**

Name: صفاء فتوح السيد			
MR. : 2126			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انها			
The patient is a 51-year-old postmenopausal individual with a history of recurrent right breast cancer. The disease was initially diagnosed in 2020, treated with surgery, radiotherapy, and systemic therapy including taxane-carboplatin and 27 cycles of trastuzumab, completed in November 2022. A PET-CT performed in April 2021 was negative for metastasis, and the patient remained disease-free until February 2024, when a recurrence was identified in the right breast without distant metastases. Subsequent treatment comprised 12 cycles of taxane, ongoing trastuzumab, and letrozole initiated in October 2024, with trastuzumab planned to continue until March 2026 (27 cycles total, pending further approval). The most recent core biopsy (i9dc) demonstrated estrogen-receptor expression of 3/8, progesterone-receptor expression of 6/8, and HER2 positivity (3+). Imaging revealed subcentimetric pulmonary nodules that were present on prior PET-CT and were non-FDG avid. A CT scan dated 30 June 2026 identified two newly developed tiny lesions measuring 7.6 mm and 5.4 mm. No other metastatic disease was identified on the latest work-up. The referring team recommended that, if the pulmonary nodules are confirmed to be non-metastatic, local therapy could be considered together with continued anti-HER2 treatment. The multidisciplinary team agreed to continue aromatase-inhibitor therapy, maintain trastuzumab supplied by a non-governmental organization, and to reassess the pulmonary nodules with imaging in three months.			
تحريراً في ,			
		التاريخ :	23-09-2026
		الوقت :	
		الطبيب المعالج :	
يعتمد ,			

Patient Name: Safaa Fattouh Al Sayed Soliman	Date : 07/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 50 Year(s)
Medical ID: 509/2021	Pat. ID: 12243

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Compared to 20.07.2024 the current study shows:

Chest:

- Rather stable course regarding the size of few right axillary level I lymph node being subcentimeteric in short axis.
- still seen left lower lung lobe atelectatic band
- Still noted right pulmonary nodules , the largest seen at right upper lobe posterior segment measuring about 7.5 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen and pelvis:

- The liver is average in size with fatty infiltration of its parenchyma. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Prominent extra renal pelvis is noted on the right side
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- The scanned skeleton shows no worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up study showing:

1. ***Rather stationary course regarding the previous study.***

DR. Prof. Ali Remah

Patient Name: Safaa Fallouh Al Sayed Soliman	Date : 01/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 50 Year(s)
Medical ID: 509/2021	Pat. ID: 12243

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Compared to 20.07.2024 the current study shows:

Chest:

- Rather stable course regarding the size of few right axillary level I lymph node being subcentimetric in short axis.
- still seen left lower lung lobe atelectatic band
- Still noted right pulmonary nodules, the largest seen at right upper lobe posterior segment measuring about 7 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes. Prominent extra renal pelvis is noted on the right side
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- The scanned skeleton shows no worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Follow up study showing:

- Overall stationary course.

Dr/Reham Khedr, MD

CT Scan
[DOCT/12243_CT_SCAN]

Patient Name: Safaa Fattouh Al Sayed Soliman	Date : 15/01/2025
Exam : MAMMOGRAPHY BOTH	Age : 50 Year(s)
Medical ID: 501/2021	Pat. ID: 12243

Combined bilateral breast mammography and complementary ultrasound

Indication:

Follow up after chemotherapy for right breast cancer

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast show subtle architectural distortion seen at UOQ about 20x15mm with underlying microcalcifications..
- No spiculated masses or suspicious microcalcifications on the left breast.
- Bilateral benign looking calcifications.
- Complementary ultrasound revealed right breast no appreciated any suspicious lesion seen as fibrotic band.
- Normal skin thickness, and nipple-areola complex of the left breast.
- No pathological right axillary or supraclavicular lymph nodes.
- Nonspecific left axillary LN.

Opinion:

- Right breast pathologically proved carcinoma on NACTH with complete radiological response (BIRADS 6)
- Benign findings left breast (BIRADS 2)

Signature Dr. Nehal Hamdy

15/01/2025 1:00 PM

Request No. : 13003475
Request Date : 11/03/2024
Verification Date : 25/03/2024
Financial Category : اورام دار السلام - ميثاره
Department : Out Patient
IHC No. :
Registered by : marwa mohamed youssef

IMMUNOHISTOCHEMISTRY REPORT

Diagnosis: Right breast mass.

size 16 cm long. Tru-cut biopsy, collected on 10/3/2024.

9. $\frac{1}{2} \ln 2$

cut from the paraffin block, processed in Bench Mark ULTRA machines (Ventana)

Sections were prepared from the paraffin block, processed in 100% ethanol and cleared in xylene. Sections were incubated with the primary antibodies using ultra view HRP DAB as chromogen and

and stained by rabbit monoclonal antibodies using

ER clone (E2), PR clone (S1)

2000

...control showed appropriate reactivity):

control showed appropriate reactivity, with a strong nuclear reaction in 8% of the tumor cells (score 3/8).

Ex: Weak nuclear reaction in 8% of the tumor cells (score 6/8).

4. **4+** Weak nuclear reaction in 70% of the tumor cells (score 3+).

Δ 50% nuclear reaction of tumor cells.

Figure 1 *Flowchart of the study*

Biopsy mass; Tru-cut biopsy:

Grade II with focal DCIS.

521

17: Positive(score 3/8).

4 pt. - Negative (score 6/8).

Strong Positive (score 3+).

positive in 50% of tumor cells.

1315/4 "ER, PR"

led by

Stefanie Möhl

Page 1 of 2

Printed By : mara mohamed yousef



Ministry of Health and Welfare
Kingdom of Saudi Arabia

Ministry of Health and Welfare
Kingdom of Saudi Arabia



MDT form

A-Patient ID:

1- Name : **مروة فتوح العبد مكرم**

Sex:

2- Medical numbers:
Female

83102059
Old Med:

509-2021

3-ID numbers:

Patient Age: 50 Y

Family History

Present

Phone

01125911101

BMI

25

REASON OF MDT

Marital status

Married

NRS Score

0/10

Group

1

Date of Mdt

11/09/2021

Co morbidity

None

Menopausal status

Postmenopausal

Initial Stage

1

Clinical data

Breast Imaging :

Mets Workup :

MBC to lung , med LN

DX
IDC

ER
3/B

PR
6/8

Her 2 Neu
3

SISH

Ki67

T

N

M

1 lung ,
med LN

Sonomammography

PATIENT HISTORY

received 15 wks taxol / carbo then kept on maintenance 27 cycles herceptin last 11/2021

pet ct 4/2021: free

ct : 9/2023: free rt axilla LN

pt is in CR &? on FU till 2/2024

recurrent rt breast mass BIRADS 5

pathology : recurrence

pet ct: newly developed rt breast small metabolic active nodule

start taxol + herceptin received 10 wks + 2 cycles

ct : free

last mdt: FOR Mammasonography then reMDT (if residual, go to surgery)

CR NO MASS

PET CT CR

MDT OUTCOME & PLAN

FOR ZOLADEX FEMARA(AFTER FSII, E2), CONTINUE IV TRASTUZUMAB

SIGNATURE

MEDICAL ONCOLOGY

Signature

RADIOLOGY

Signature

SURGERY

Signature

PATHOLOGY

Signature